

IMPACT OF SPECIALTY PHARMACY INTEGRATION ON TIME TO MEDICATION ACCESS FOR PRESCRIPTION CANNABIDIOL

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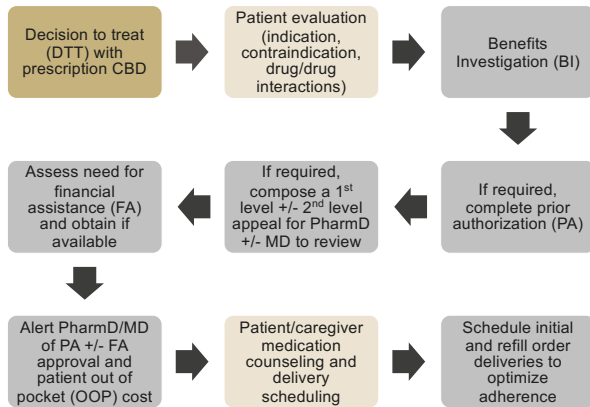
BACKGROUND

- Access to prescription cannabidiol (CBD), an adjunct therapy for uncontrolled seizure disorders, is restricted by insurance requirements which may potentially lead to initiation delays.¹
- Integrated specialty pharmacies (ISPs) composed of pharmacists and certified pharmacy technicians (CPHT) can navigate the cumbersome medication access pathway to assist in patients initiating prescription CBD in a timely manner.

STUDY OBJECTIVE

Measure the duration from initial specialty pharmacy patient assessment (when embedded pharmacy team is notified of decision to treat) to prescription CBD access.

Figure 1. Integrated Specialty Pharmacy Workflow



Healthcare Team Member:

MD PharmD CPhT

METHODS

Design	Single-center, retrospective cohort study
Inclusion	All patients prescribed CBD by center's outpatient neurology clinics between January 2019 – April 2020
Exclusion	- Participation in a prescription CBD clinical trial - Access and fulfillment process not handled by ISP
Outcomes	- Primary: Interval from initial patient assessment to first prescription CBD shipment in days - Secondary: Interval from first assessment to BI, BI to insurance approval, insurance approval to initial shipment, and FA impact on OOP cost
Data sources	Electronic health record and specialty pharmacy management system

RESULTS

Figure 2. Embedded Clinic CPhT Role

Pre-Prescription CBD Launch	- Coordinate with clinical staff to discuss and initiate standardized access process - Interface with market access representatives to discuss potential obstacles and FA options
Patient Prescription CBD Initiation	- Complete pharmacy BI - Complete PAs and appeals - Evaluate patient OOP cost and obtain FA if needed
Ongoing Treatment Access and Coordination of Care	- Ensure timely renewal of PAs - Monthly check-in to schedule refill delivery and assess for medication changes and seizure control - Maintain and renew patient FA as required

Table 1. Cohort Demographics (N = 136)

	Pediatric (N=92) % (n)	Adult (N=44) % (n)
Age, years [median, (IQR)]	10 (5 – 14)	28 (21 – 44)
Gender, female	47 (43)	57 (25)
Race, white	84 (77)	86 (38)
Insurance type		
Medicaid	73 (67)	32 (14)
Commercial	20 (18)	23 (10)
Medicare	--	46 (20)
TriCare	5 (7)	--
Height, cm [median, (IQR)]	130 (102 – 147)	164 (153 – 173)
Weight, kg [median, (IQR)]	29 (17 – 38)	62 (49 – 76)
Lennox-Gastaut Syndrome	89 (82)	80 (35)

IQR = interquartile range

Figure 3. Time From Initial Patient Assessment to Prescription CBD Shipment (N = 136)

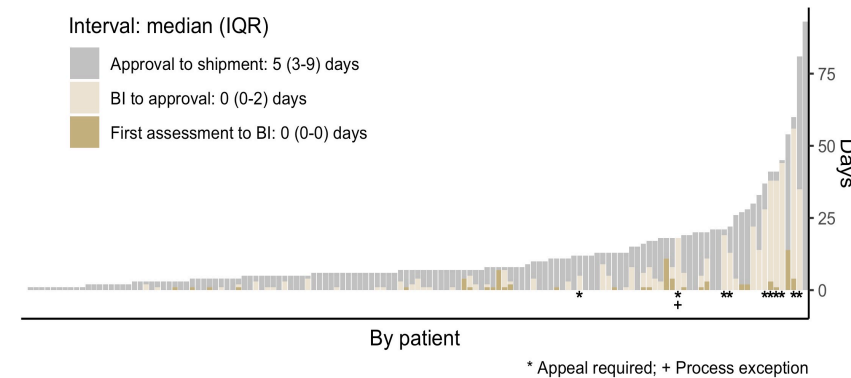
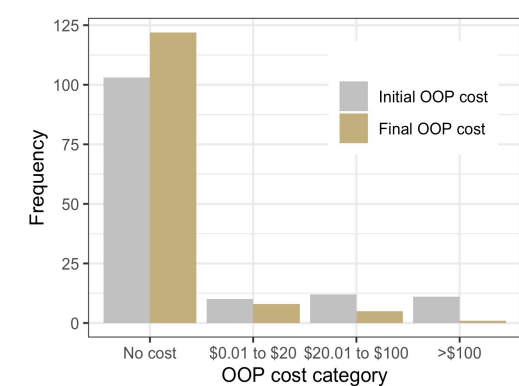


Figure 4. Financial Assistance Impact on Initial OOP Cost



- 80 (59%) patients were able to start therapy within 7 days or less from initial assessment
- Process exception patient paid cash for immediate treatment initiation while ISP obtained medication approval
- Median time from DTT to initial medication shipment was 7 days (IQR 4 – 13)

CONCLUSIONS

- The management of a prescription CBD approval pathway by an integrated specialty pharmacy team ensures timely access to therapy
- Certified pharmacy technician ownership of key steps in the prescription CBD approval pathway under pharmacist oversight streamlines access and allows the pharmacist to focus on clinical aspects of patient care

- A total of 14 patients (10%) required FA
- After FA, 130 of the 136 (96%) patients had an OOP ≤ \$20.00 on initial fill